INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pemetrexed + Cisplatin

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00317
Version 7

Pemetrexed and cisplatin are given by drip on Day 1, then repeated every 21 days (3 weeks), if your team confirms it is safe.

Treatment stages

01 **Day 1 chemotherapy, then recovery and review**
Two infusions every three weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Pemetrexed + Cisplatin
National Cancer Control Programme (NCCP) 00317

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe or persistent diarrhoea, inability to drink, very little urine, a spreading or blistering rash, or swelling/wheeze during an infusion. Tell the team promptly about new tinnitus, hearing change, numbness, weakness or difficulty walking.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your two chemotherapy medicines

Pemetrexed + Cisplatin is chemotherapy used for selected people with unresectable pleural mesothelioma or non-squamous non-small-cell lung cancer. The aim may be disease control or, within a separate combined preoperative plan, support for surgery. Ask your team to explain the expected benefit for your own diagnosis.

ANTIFOLATE CHEMOTHERAPY

Pemetrexed

Alimta



Interrupts deoxyribonucleic acid (the cell's genetic material) (DNA) building-block production

Pemetrexed blocks several enzymes that use folate to make the building blocks of DNA and RNA. This can slow or stop cancer-cell growth. Healthy cells can also be affected. Prescribed folic acid and vitamin B12 reduce important toxicities and are part of treatment.

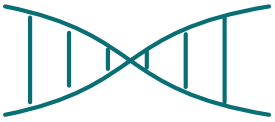
Folate-dependent enzymes blocked → DNA building blocks reduced → Cell growth disrupted

What to watch for

Take the prescribed folic acid and attend for B12. Report mouth sores, diarrhoea, a rash, breathlessness or reduced urine; check before taking ibuprofen or other non-prescription medicines.

PLATINUM CHEMOTHERAPY

Cisplatin



Damages DNA through platinum links

Cisplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing, but healthy tissues may also be affected. Kidney protection with fluids and monitoring is central to cisplatin treatment.

Platinum reaches cells → DNA links form → Cell division disrupted

What to watch for

Report ringing in the ears, hearing changes, numbness or loss of function. Tell the team if you cannot drink, are passing less urine or become swollen or breathless during fluids.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Evidence depends on the cancer and the exact medicines

Ask which study and treatment aim match your situation.

In unresectable pleural mesothelioma, a randomised study found better survival with pemetrexed plus cisplatin than cisplatin alone. A different advanced non-small-cell lung cancer (NSCLC) trial compared cisplatin/pemetrexed with cisplatin/gemcitabine; benefit differed by tumour histology, supporting non-squamous selection. These studies cannot predict an individual outcome, and neither describes standalone neoadjuvant immunotherapy.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

A 21-day chemotherapy cycle within your agreed course

Pemetrexed + Cisplatin

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TWO INFUSIONS EVERY THREE WEEKS

Day 1 chemotherapy, then recovery and review

D1 | D2–D4 | D5–D20 | D21

Both chemotherapy medicines are planned on Day 1 of a 21-day cycle. For pleural mesothelioma, the source describes up to 4–6 cycles. For advanced non-squamous non-small-cell lung cancer (NSCLC), it describes 4–6 combination cycles followed, when appropriate, by a separate pemetrexed maintenance plan (NCCP 00222). The selected preoperative nivolumab combination uses three cycles and a separate immunotherapy plan (NCCP 00849). Your team confirms which course applies and may stop or change it earlier.

For pleural mesothelioma, the source describes up to 4–6 cycles. For advanced non-squamous NSCLC, it describes 4–6 combination cycles followed, when appropriate, by a separate pemetrexed maintenance plan (NCCP 00222). The selected preoperative nivolumab combination uses three cycles and a separate immunotherapy plan (NCCP 00849). Your team confirms which course applies and may stop or change it earlier.

BASELINE → TREATMENT → FOLLOW-UP

Checks from preparation through the next treatment plan

These columns show the order of care, not equal time intervals. Baseline hearing tests are only if indicated; blood counts, kidney and liver tests repeat before each cycle. Any additional immunotherapy has its own monitoring plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00317 v7 · official regimen

THE WHOLE TREATMENT JOURNEY

Pemetrexed + Cisplatin: treatment and checks

Pleural mesothelioma / non-squamous NSCLC · chemotherapy component

Both chemotherapy medicines are planned on Day 1 of a 21-day cycle. For pleural mesothelioma, the source describes up to 4–6 cycles. For advanced non-squamous NSCLC, it describes 4–6 combination cycles followed, when appropriate, by a separate pemetrexed maintenance plan (NCCP 00222). The selected preoperative nivolumab combination uses three cycles and a separate immunotherapy plan (NCCP 00849). Your team confirms which course applies and may stop or change it earlier.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each chemotherapy cycle	Between visits	Response / course review	Completion / next plan
Chemotherapy course NCCP 00317	Confirm disease and course	Day1 pemetrexed then cisplatin;21-day cycle	Recovery, daily folate and symptom reporting	Mesothelioma 4–6; advancedNSCLC4–6; selected neoadjuvant3	Separate maintenance/ surgery/ follow-up plan
Blood count, kidneys and liver NCCP 00317	Baseline full blood count (FBC), renal and liver tests	Repeat before each cycle	Earlier tests if illness or dehydration develops	Review changes before continuation or dose modification	Follow unresolved abnormalities
Hydration and electrolytes NCCP 00317	Assess renal/ cardiac status and local fluid plan	Supervised pre/post-cisplatin fluids; salts as prescribed	Report poor intake, reduced urine, swelling or breathlessness	Reassess fluid and dose plan if kidneys change	Follow persistent kidney/ electrolyte problems
Nerves, hearing and symptoms NCCP 00317	Record baseline nerve/hearing problems; audiology if indicated	Review toxicity and function before each dose	Report symptoms promptly; use urgent contact plan	Balance benefit and lasting side effects	Review persistent neuropathy, hearing or kidney problems
Vitamins and prevention medicines NCCP 00317	Folate preparation; B12 in preceding week; clear steroid plan	Folate daily; B12 every three cycles; reconcile prevention medicines	Follow written plan; report missed doses or poor intake	Reconcile medicines if platinum stops or phase changes	Folate to 21 days after last pemetrexed; separate plan if maintenance continues
Disease assessment NCCP 00317	Confirm diagnosis, intent and selected indication	Assess symptoms and clinical progress	Arrange extra assessment if indicated	Scans/tests according to consultant plan	Agree surveillance, surgery review or separate maintenance

What each check is for

1 **Before first pemetrexed**

In the week before treatment

Vitamin preparation reduces serious toxicity.

Ask / act: Confirm preparation and a single reconciled steroid prescription.

2 **Before every cycle**

Before dose authorisation

Counts, organ function and toxicity can change.

Ask / act: Review results before deciding continue, delay or modify.

3 **At the end of combination treatment**

At the indication-specific course review

Maintenance and preoperative plans differ.

Ask / act: Confirm the next regimen and which supportive medicines continue.

4 **After last pemetrexed**

Through the following21days

Vitamin protection extends beyond the final infusion.

Ask / act: Confirm stop/review dates with the team; request follow-up for persistent symptoms.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the visit

Confirm blood-test arrangements, transport, prescriptions, B12 booking and the 24-hour contact number. Tell the team if you are ill before travelling.

STEP 2

Time in the unit

Pemetrexed is given into a vein over about 10 minutes, followed by cisplatin over about 60 minutes, starting 30 minutes after pemetrexed. Fluids before and after cisplatin make the visit longer; the team individualises hydration and electrolyte replacement. The whole visit also includes blood-result review, prevention medicines, preparation and observation.

STEP 3

Access for treatment

Treatment may use a small cannula or an existing suitable central line. The team chooses access for your veins and treatment plan. There is no routine take-home infusion pump in this regimen; ask for line-care instructions if you have a line.

STEP 4

Home medicines

Keep your vitamin, steroid, anti-sickness and rescue plans together. Ask the pharmacist to resolve duplicate dexamethasone directions before taking them.

STEP 5

Support and review

Arrange help with travel and daily tasks if needed. Bring symptom notes, a medicines list and questions to review. Request more passport pages rather than treating the printed page count as a course limit.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Chemotherapy	 **Day 1** Both infusions on Day 1 after review
Prevention medicines	 **Days 1–4** Follow the written anti-sickness and steroid plan; folic acid continues every day
Symptoms can occur	 **Days 1–21** Report symptoms throughout the cycle; this bar does not measure risk
Next cycle review	 **Days 20–21** Blood tests and symptom review before the next dose; appointments may differ

Before starting: Folate and B12 preparation begins before the first dose; confirm the Day –1 rash steroid.

Day 1 and early days: Both infusions are planned on Day 1. The written anti-sickness schedule extends over the following days; folate continues daily.

No safe-day prediction: Low counts or serious symptoms can occur between visits. The position in the cycle never rules out infection or another urgent problem.

Across the course: Nerve, hearing and kidney effects can accumulate or persist. The number of cycles and any transition to maintenance depend on the indication and consultant review.

Why these bands stay broad: This is one reference cycle, not a prediction of when side effects will happen. Risks overlap and symptoms can persist after the last dose. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00317 v7 · official regimen ↗

There are no guaranteed "safe days". Follow the temperature threshold and emergency instructions given by your unit. Sudden illness, shaking chills or feeling very unwell need urgent advice even without a high temperature.

Between-treatment notes

Day 1: Treatment day

Pemetrexed is given into a vein over about 10 minutes, followed by cisplatin over about 60 minutes, starting 30 minutes after pemetrexed. Fluids before and after cisplatin make the visit longer; the team individualises hydration and electrolyte replacement. The whole visit also includes blood-result review, prevention medicines, preparation and observation. Tell the nurse immediately about rash, swelling, chest tightness, dizziness, pain or stinging near the drip.

Day 2–4: Follow your home prevention plan

Continue folic acid and the prescribed anti-sickness medicines. Your team combines the pemetrexed rash steroid and anti-sickness steroid into one clear schedule. Do not double up dexamethasone courses. Call if vomiting prevents medicines or fluids staying down.

Day 5–20: Recovery and symptom monitoring

Fatigue, bowel or mouth symptoms and low blood counts may develop between visits. Nerve or hearing symptoms can build across cycles. Use the emergency contact plan at any time; do not wait for the next treatment day.

Day 21: Review before the next cycle

The team checks blood counts, kidney and liver results and symptoms before deciding whether to continue, delay or change a dose. Bring your medicine list and symptom notes; confirm the next B12 injection and whether the treatment phase changes.

Support around the medicines

1 **Folic acid every day**

The source uses 350–1000 micrograms daily: at least five doses during the seven days before the first pemetrexed, throughout treatment and for 21 days after the last pemetrexed. Follow your own prescription and tell the team about missed doses.

2 **Vitamin B12 injection**

The source uses 1000 micrograms into a muscle during the week before the first pemetrexed, then every three cycles. Later injections can be on treatment day. Confirm who books them.

3 **One steroid schedule**

Pemetrexed rash prevention uses dexamethasone-equivalent 4 mg twice daily on the day before, day of and day after treatment. The anti-sickness plan also uses steroids. Your prescriber/pharmacist must provide one reconciled schedule; do not add two courses together.

4 **Preventing and treating sickness**

This cisplatin combination has high sickness risk. Take prevention medicines as prescribed even when you feel well, and keep the team’s rescue plan available. The usual high-risk reference includes olanzapine; ask about sleepiness, falls and interactions.

5 **Fluids and kidney protection**

Ask for an individual drinking plan, especially with heart or kidney problems. Report inability to drink or reduced urine. Cisplatin needs supervised fluids before and after treatment; additional salts may be prescribed.

6

Medicines and self-care

Check all prescriptions, painkillers, supplements and vaccines with the oncology team. Some anti-inflammatory medicines can affect pemetrexed clearance; do not start or stop them on this guide's authority. Follow mouth-care and infection advice.

Understanding your treatment

The same chemotherapy pair appears in more than one disease setting.

This guide's scope

This summary covers the two chemotherapy medicines. If nivolumab is part of your treatment before surgery, use the full nivolumab combination plan. The nivolumab combination has its own side effects, monitoring and urgent advice; it is not covered by a chemotherapy-only handout. Ask your team to confirm whether the plan is for mesothelioma, advanced non-squamous lung cancer or treatment before surgery.

Before the first treatment

Preparation reduces avoidable risk and makes the plan clearer.

Blood tests and kidney assessment

Baseline checks include full blood count, kidney and liver tests. The team also reviews the cisplatin hydration plan and existing kidney problems.

Nerves and hearing

Describe any numbness, weakness, ringing in the ears or hearing difficulty before starting. A hearing test is arranged when clinically indicated.

Vitamins, steroids and medicines

Confirm that folic acid has started, B12 was given, and you have one clear steroid and anti-sickness prescription. Bring a complete medicines list.

Consent and priorities

Confirm the treatment aim and indication, likely benefit and uncertainty, reasonable alternatives including not proceeding, serious and lasting risks, kidney/hearing/nerve problems, fertility and pregnancy advice, vitamin and steroid plans, and the chance of delays or dose changes.

What happens on treatment day

The infusion times are only part of your visit.

Assessment first

Blood results and symptoms are reviewed before a dose is authorised. A delay or reduction may allow recovery and reduce lasting harm.

During the infusion

Tell the nurse immediately about feeling unwell, breathing difficulty, itching, swelling, pain, burning or redness around the drip. Do not wait to see whether it settles.

Looking after yourself between visits

Ask for help early with symptoms and practical difficulties.

Symptoms and daily life

Rest when needed while keeping activity manageable. Ask for nutrition, fatigue, work, transport or financial support. Keep a short symptom record for review.

Mouth and bowel care

Follow the team's mouth-care plan. Report painful swallowing, persistent diarrhoea, vomiting or difficulty maintaining fluids; severe symptoms need urgent advice.

Hearing and nerves

Report tinnitus, hearing change, numbness or difficulty with everyday tasks before further treatment. These effects can persist after treatment ends.

Fertility, pregnancy and breastfeeding

Discuss fertility preservation, contraception and pregnancy plans before treatment. Pregnancy and breastfeeding are exclusions in the regimen. Ask the team for the medicine-specific contraception period.

When to get urgent help

Use your unit's 24-hour advice number and emergency instructions.

Possible infection

Call immediately for the fever threshold your unit gives you, shaking chills or sudden illness. You can have a serious infection without a high temperature. Do not wait for the next appointment.

When you call

Give your medicines, last treatment date and symptoms. Explain whether you can drink and pass urine. If emergency help is needed, follow local emergency instructions and tell staff you are receiving chemotherapy.

Review, completion and a possible next phase

The plan changes according to benefit, recovery and your priorities.

Response assessment

Scans and other tests are timed by the consultant's treatment plan. Symptoms and side effects alone do not show whether treatment is working.

After the last pemetrexed

Continue folic acid for 21 days after the final dose. Agree follow-up for persistent kidney, blood-count, hearing, nerve or other problems. A maintenance prescription has its own ongoing tests and prevention medicines.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber’s review. The patient page explains the checks and does not authorise treatment.

Is immunotherapy part of these two medicines?

This guide covers pemetrexed and cisplatin chemotherapy. It does not include an immunotherapy medicine. If nivolumab, pembrolizumab or another medicine is also prescribed, obtain its separate regimen, consent, monitoring and emergency instructions. Mesothelioma and NSCLC have different aims and evidence; ask which indication is on your prescription.

Why take vitamins during chemotherapy?

Folic acid and vitamin B12 lower important pemetrexed side effects. They are prescribed parts of this regimen. Tell the team promptly if they were missed.

What happens when the platinum chemotherapy stops?

The next step depends on your disease and treatment aim. Selected advanced NSCLC plans may move to pemetrexed maintenance under a separate regimen. Preoperative treatment leads to a surgical-team review; mesothelioma follow-up is agreed with the oncology team.

What should I be able to explain before I leave?

Ask your nurse or doctor to check that you can explain which medicines you are receiving, the treatment days, which medicines to take at home, and which number to ring urgently. Ask for the plan in writing if any part is unclear.

> **Try explaining the plan back:** Can you describe which two chemotherapy medicines you are receiving, when to take folic acid and steroids, when your B12 injection is due, and which number to call at any hour?

Which treatment this guide covers This guide covers pemetrexed and cisplatin chemotherapy. It does not include an immunotherapy medicine. If nivolumab, pembrolizumab or another medicine is also prescribed, obtain its separate regimen, consent, monitoring and emergency instructions. Mesothelioma and non-small-cell lung cancer (NSCLC) have different aims and evidence; ask which indication is on your prescription.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pemetrexed + Cisplatin · chemotherapy day

Active medicines: pemetrexed + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V8 row Reference course only: reconcile with pemetrexed rash prophylaxis and aprepitant interaction into one unambiguous prescription. Do not add both courses.
Olanzapine	5 mg orally once daily	Day1 and Days2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose

Medicine	Dose / route	When
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.

Additional phase-specific support

- High emetic risk driven by cisplatin. Use general high-risk NCIS V8 row.
- Reconcile pemetrexed rash and antiemetic dexamethasone into one prescription.
- Folic acid daily and B12 every three cycles; continue folate21days beyond the last pemetrexed.
- Cisplatin requires pre/post hydration and electrolyte review under local policy.
- An additional nivolumab or other drug needs its own regimen and support plan.
- When platinum stops or another phase begins, review the antiemetic plan; do not carry the entire high-risk course into pemetrexed-only maintenance automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Pemetrexed + Cisplatin

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Ears / hearing

Cisplatin

Report tinnitus or hearing change promptly; describe any existing problem before starting.

2 Mouth

Pemetrexed

Mouth ulcers or painful swallowing can limit eating and drinking. Seek advice early.

3 Chest / breathing

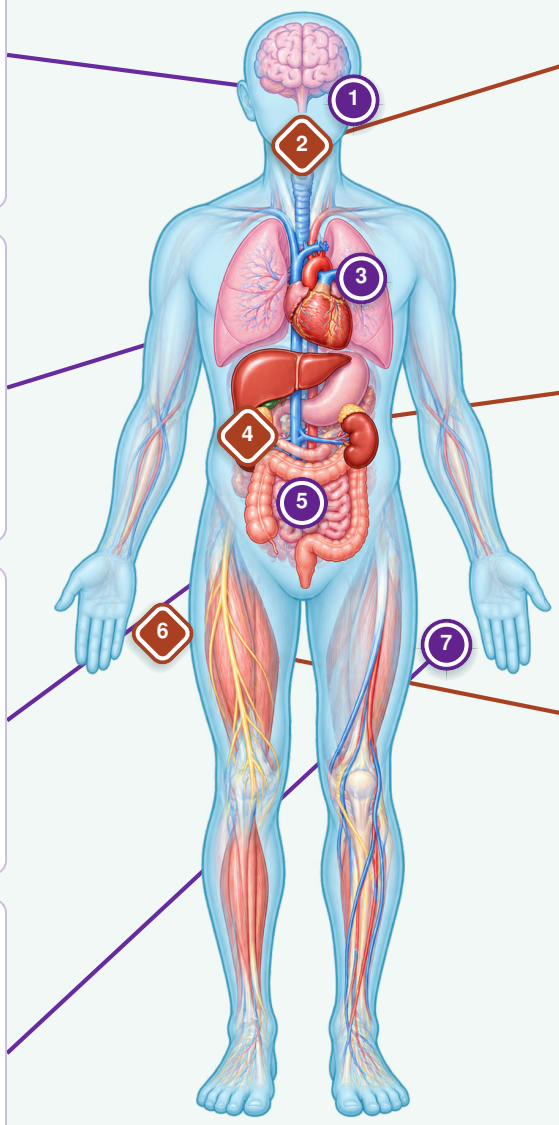
Both chemotherapy medicines

New breathlessness, wheeze or chest pain needs urgent assessment; do not assume it is an expected side effect.

4 Kidneys / fluids

Both chemotherapy medicines

Report reduced urine, inability to drink, swelling or dehydration. Kidney tests guide treatment.



5 Stomach / bowel

Both chemotherapy medicines

Use prescribed anti-sickness medicines. Severe vomiting, diarrhoea or poor intake needs urgent advice.

6 Hands / feet

Cisplatin

Report tingling, numbness or difficulty with buttons, balance or walking; symptoms can persist.

7 Skin / blood counts

Both chemotherapy medicines

Report a spreading rash, unusual bruising or bleeding. Fever or sudden illness needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00317 v7 · official regimen**
   https://healthservice.hse.ie/documents/6922/317_V7_PEMEtrexed_and_CISplatin.pdf
2. **EMA ALIMTA product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/alimta-epar-product-information_en.pdf
3. **HSE chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
4. **NCCP NCIS Medical Oncology antiemetics V8, Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Vogelzang 2003: pemetrexed/cisplatin versus cisplatin in mesothelioma**
   https://pubmed.ncbi.nlm.nih.gov/12860938/
6. **Scagliotti 2008: cisplatin/pemetrexed versus cisplatin/gemcitabine in advanced non-small-cell lung cancer (NSCLC)**
   https://pubmed.ncbi.nlm.nih.gov/18506025/
7. **CheckMate816: nivolumab plus chemotherapy; separate combination evidence**
   https://pubmed.ncbi.nlm.nih.gov/35403841/